

Intermittent Self-Catheterization (ISC)

Patient Information · Emptying the bladder yourself with a thin catheter

WARWIKI

Intermittent self-catheterization (ISC) is a way to empty your bladder yourself. You gently pass a thin tube (catheter), let the urine drain, and take it out. You do this a few times a day. It is used when the bladder does not empty well on its own. Once you learn it, it takes only a few minutes.

Why It's Done

The bladder may not empty fully after some surgeries, with a **nerve-related bladder problem**, after bladder reconstruction, or because of a blockage. Urine left behind can lead to infections and leaking, and high pressure can harm the kidneys. ISC **empties the bladder** on a schedule.

Why Not a Catheter That Stays In?

- Guidelines prefer ISC to a left-in (indwelling) catheter when you can do it safely.
- **No bag.** Nothing is attached between times.
- You will still need regular check-ups of your bladder and kidneys.

It can feel scary at first. Most people, and caregivers, learn it quickly with a nurse's help. A different catheter or a handling aid can help if your hands are weak.

Learn the Terms

ISC

Emptying the bladder yourself with a catheter, then taking it out.

Catheter

A thin, soft tube passed into the bladder to drain urine.

Residual urine

Urine left in the bladder after you urinate.

Neurogenic bladder

A bladder that does not work normally because of a nerve problem.

Clean technique

Washing your hands and the area. It does not need to be sterile.

Indwelling catheter

A catheter left in place with a bag.

Hydrophilic catheter

A catheter with a slippery coating that makes it easier to pass.

Does it hurt? Usually not. Most people feel only mild pressure. Catheters are thin and slippery or come with gel. A little discomfort or spotting is common the first few times. A nurse teaches you step by step.

How to Do It

- 1 **Wash your hands**, get your supplies ready, and clean the opening as you were taught.
- 2 **Gently insert** the catheter until urine flows, then just a little farther. **Never force it.** Stop if you feel strong resistance, pain or new bleeding.
- 3 Let the bladder **drain fully**. Pull the catheter out slowly, pausing if more urine comes.
- 4 Throw away a single-use catheter. Clean a reusable one only if it is labeled for reuse, as taught. Wash your hands.

How Often

- Often **4 to 6 times a day**. Your team sets your schedule.
- Do not skip times. The aim is to keep your bladder from getting too full.
- Record how much you drain if you are asked to.

Good to Know

- Bacteria in the urine are common and are treated only if you have **symptoms**.
- About 1 in 4 people who use ISC get repeated infections. Your team can review your routine and your options.
- A caregiver can be taught if you cannot do it yourself.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, chills, or side or back pain
- Bleeding that does not clear, or pain that keeps getting worse
- New trouble passing a catheter that used to go in easily
- New leaking between catheterizations

Go to an emergency department if:

- You cannot pass the catheter and your bladder feels full or painful
- You have a spinal cord injury and get a pounding headache, flushing or sweating
- You cannot reach the office and are worried

Three Things to Remember

1. ISC empties a bladder that cannot empty well on its own.
2. Keep to your schedule and use clean technique.
3. Never force the catheter.